



Medication: Zuclopenthixol Acetate (Clopixol Acuphase®) (Mental Health) Procedure

ALERT • Do not confuse with Zuclopenthixol Decanoate (Clopixol Depot®) a long – acting intramuscular (IM) injection which is used as long term maintenance treatment in chronic psychoses.

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

General information

- Zuclopenthixol Acetate (Clopixol Acuphase®) is an intermediate-acting antipsychotic injection indicated for acute psychosis and acute mania where other treatment modalities have been ineffective.
- There are a number of precautions, contra-indications and side effects that the medical team will need to consider prior to prescribing Zuclopenthixol Acetate.
- Additional attention needs to be given to monitoring of patients, post injection, by the nursing team at an agreed minimum standard.
- Sedation, extrapyramidal side effects, (EPSE) hypotension and tachycardia are commonly encountered adverse effects of Zuclopenthixol Acetate. Sedation can be expected to occur in the initial 2-3 hours post dosing and be maximal at 12 hours, but this pattern may vary. The effects of Zuclopenthixol Acetate can persist for up to 72 hours, though levels can still be detected in the serum after this time. Care should therefore be exercised if additional doses of antipsychotics or other medications with central nervous system (CNS)/respiratory depressant effects (e.g. benzodiazepines, opioids) are prescribed.
- Zuclopenthixol Acetate can also cause QT prolongation, so the risks and benefits of treatment should be carefully considered when prescribing concomitant medications

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Feb 2018	Revisions	NA	National Standards 1, 4, 5, 6
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that can also have this effect (e.g. other antipsychotics, methadone and macrolide antibiotics). A baseline ECG should be obtained if possible as well as a follow-up ECG.

- Anticholinergics such as Benztropine^{3,4} should be prescribed in the PRN section of the HMC to manage potential EPSE and nursing staff should discuss this with the medical prescriber. The prescriber must document in the patient's progress notes that they have reviewed the above and any additional instructions that may apply in the 72 hours post Zuclopenthixol Acetate (e.g. whether a certain drug needs to be withheld).
- In the instance where there is no documentation evident, nursing staff must contact the prescriber (or duty officer/on call Consultant, if after hours) to review the above before additional antipsychotics, benzodiazepines and/or sedating medications can be given within this specified period.

Procedure

- The first injection is usually the most sedating. Sedation may initially be seen within 2 hours after injection, peaks after 12–36 hours and may last up to 3 days. The following must be considered:
 - * Patients **must** be carefully monitored following each injection.
 - * Physiological observations must be recorded on the Adult Observation and Response Chart (ADDS) chart (RGMR141.A) with an update to the Psychiatric Services Online Information System (PSOLIS) care plan regarding frequency of observations.
 - * If the patient is fully alert and ambulating but uncooperative and physiological observations cannot be completed, then it must be documented that the patient was visually assessed as observations refused by patient. The table below indicates the recommended frequency for vital observations post Zuclopenthixol Acetate. Vital signs may be completed more frequently as per the medical officer (MO) discretion or as clinically indicated. This may be particularly relevant at 12 hours post Zuclopenthixol Acetate or at 24-36 hours post dose (corresponding with peak serum levels).^{3,4}
 - * When patients are asleep (e.g. during the night) their respiratory function should be monitored on an hourly basis which involves monitoring the patient's respiratory rate (RR), rise and fall (depth) of chest, the presence of any sounds suggestive of laboured, obstructed or impaired breathing (e.g. wheezing, snoring).
- Attempts should be made to wake the patient if their RR is less than 10 breaths per minute, or if there is any evidence of respiratory distress or obstruction. A [Code Blue Medical Emergency](#) should be initiated if there is difficulty arousing the patient as per existing policy.
- Patient observations must be recorded throughout the night.

Patient monitoring

- Prior to injection, baseline temperature, heart rate (HR), blood pressure (BP), RR, oxygen saturations and ECG should be performed as is considered best practice.²
- Document if unable to be completed due to agitated state of the patient.

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- Monitoring is of particular importance after the onset of sedative action occurs at 2 hours and peaks at 12 hours post injection.
- After the first injection, vital signs should be monitored every 15 minutes for the first hour, then hourly for the next 4 hours.
- Monitoring should include observation for excessive sedation, snoring, pyrexia, muscle rigidity or other EPSE.
- Thereafter, vital signs should be monitored every 2 hours for the next 6 hours, coinciding with the anticipated period of maximum sedation.
- If vital signs remain stable, observations can be taken 4 hourly for the period up to 36 hours.
- Take ECG at 24 hours post injection to ensure safety of the QT interval,
- If vital signs become unstable medical staff should be contacted.
- Document all observations on the ADDS chart (RGMR 141.A).
- Refusal of observations must be clearly documented each time
- Document if the patient is aggressive and it is unsafe to complete physical observations.

Monitoring Guidelines	
T, HR, BP, RR, O ₂ Sats	
ECG	
Baseline prior to injection	
Post injection 15 minutes	
30 minutes	
45 minutes	
1 hour	
2 hours	
3 hours	
4 hours	
6 hours	
8 hours	
12 hours	
16 hours	
20 hours	
24 hours	
ECG	
28 hours	
32 hours	
36 hours	

Compliance monitoring

Medication Datix incidents will be reviewed.

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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. 2017. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>
2. Kapur S, Paton C, Taylor D. The Maudsley Prescribing Guidelines in Psychiatry (12th ed). 2012. New Delhi: Wiley-Blackwell.
3. MIMS Online. Clopixol Product Information. 2017. Available from https://www-mimsonline-com-au.smhslibresources.health.wa.gov.au/Search/FullPI.aspx?ModuleName=Product Info&searchKeyword=clopixol&PreviousPage=~ /Search/QuickSearch.aspx&SearchType=&ID=32810001_2
4. Therapeutic Guidelines Pty Ltd. Psychotropic Guidelines. 2017. Available from <https://tgldcdp-tg-org-au.smhslibresources.health.wa.gov.au/topicTeaser?guidelinePage=Psychotropic&etgAccess=true>

Links

[ACSQH: Medication Safety Standard 4.1](#)

[DoH High Risk Medication Policy](#)

[RKPG Medication Administration Clinical Practice Standard](#)

Acknowledgement

We acknowledge the following previous site endorsed work and/or contributors used to compile this policy:

- Fiona Stanley and Fremantle Hospital and Health Service, Guidelines for the use of Zuclopenthixol Acetate® (Acuphase®) issued Jan 2015 Liana Shymko - Senior Pharmacist, Alma Street Centre FHHS
- North Metropolitan Health Service, Mental Health. Area Mental Health Policy Manual – Zuclopenthixol Acetate (Clopixol Acuphase®): Prescribing Guidelines Issued 22 May 2013. NMHS Mental Health Executive Group

National Standards



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